

Influence of a Nursing Intervention On Regimen Adherence and Societal Adjustments Postmyocardial Infarction

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One hundred three first-time myocardial infarction (MI) patients were randomly assigned to either an experimental or a control condition. Patients completed a cardiac rehabilitation program during hospitalization and were interviewed to assess intentions to follow regimen prescriptions, attitudes toward the prescriptions, coping methods, and the perceived beliefs of others concerning their intentions. Patients were visited at home 30 days after discharge and reassessed on each of the above variables except that their behavior was substituted for intentions and societal adjustment was assessed. The experimental group was given an intervention program which included a discussion of assessment data, identification of problems, and establishment of goals. The assessment was repeated 60 days after discharge. No differences were found between experimental and control groups for either medical regimen adherence or societal adjustment. There was a significant decrease in mean scores for all variables from hospital to 30 days for both groups, but no change from 30 to 60 days. Attitudes and perceived beliefs of others were predictive of adherence, and it was concluded that these variables need to be included in any rehabilitation program.

Health care providers have assumed responsibility for facilitating rehabilitation for individuals with heart disease through special programs designed to increase understanding of cardiac risk factors and through interventions that implement and reinforce preventive behaviors. Despite the knowledge gained from rehabilitation instructions, adherence behaviors over time decline after patients return to a societal role (Bengtsson, 1983; Hentinen, 1986; Oldridge et al., 1983). In addition, there is little evidence of the type of program intervention needed to enhance adherence or societal adjustment on a long-term basis for myocardial infarction (MI) patients. Psychosocial factors have been identified that affect health promotion behavior (Pender & Pender, 1986) and regimen adherence and social adjustment postinfarction (Dhooper, 1984; Mayou, 1984), yet these factors are not consistently included in rehabilitation programs.

Theoretical Framework

The Fishbein model of reasoned action as described by Ajzen and Fishbein (1980) was used as the theoretical basis for this study to examine the effect of a nursing intervention on medical regimen adherence (MRA) and societal adjustment (SA) postinfarction. According to the Fishbein model, individuals' beliefs serve as the information base for their attitudes, intentions, and behaviors. As applied to this study, the MI patient's beliefs (information about the heart and medical regimen) would influence the development of attitudes (favorable or unfavorable). These attitudes, combined with perceived beliefs of others (patient's perceptions of others' beliefs [PBOs] about adherence and SA), would influence the formation of intentions and determine the patient's behavior (MRA and SA).

Attitudes, intentions, and PBOs have been found to be predictive of behavior in the sociological realm (Ajzen & Fishbein, 1980; Ajzen & Madden, 1986; Jaccard & Becker, in press). Attitudes were also found to be positively related to health promotion behavior by Pender and Pender (1986), to societal adjustment postinfarction with MI patients by Wiklund, Sanne, Vedin, and Wilhelmsson (1984), and to medical regimen adherence by Miller, Johnson, Wikoff, McMahon, and Garrett (1983).

Perceived beliefs of others have been found to influence societal adjustment in MI patients by Doherty, Schrott, Metcalf, and Issello-Vailes (1983); Miller, Johnson, Garrett, Wikoff, and McMahon (1982a); and Miller, Wikoff, McMahon, Garrett, and Ringel (1985b). In addition to patients' PBOs, tangible support by spouses was found to promote maintenance of adherence behaviors postinfarction by Holm, Fink, Christman, and Ashley (1985); Doherty et al. (1983); Mayou (1984); and Oldridge and Jones (1986). Dhooper (1984) also found family and friends' emotional support for the MI patient aided societal adjustments postinfarction.

In addition to the variables of the Fishbein model, identification of coping methods was determined to be important, as coping methods relate directly to strategies used by the individual to deal with difficulties of MRA and SA. An individual's coping methods have been found to be related to MRA and SA in hypertensive patients by Jalowiec and Powers (1981), and in burn patients by Roberts et al. (1987). Miller, Garrett, McMahon, Johnson, and Wikoff (1985a) reported that both active and passive coping methods were used by MI patients, and Dinsdale and Hackett (1982) found adjustments postinfarction were promoted by denial.

Difficulty in adherence arises because of the varied sit-

uations and settings where regimen prescriptions must be followed. Taylor, Houston-Miller, Ahn, Haskell, and Debusk (1986) found different settings and situations affected MRA and SA by MI patients. McMahon, Miller, Wikoff, Garrett, and Ringel (1986) reported home as the setting of greatest adherence and work the setting of least adherence.

Both written and oral nursing interventions posthospitalization have been found to be effective in maintaining or increasing adherent behavior and were viewed positively by the patient (Garnett, Davis, McKenney, & Steiner, 1981; Glanz, Kirscht, & Rosenstock, 1981; Kirscht, Kirscht, & Rosenstock, 1981). However, Kirscht et al. found inconsistent or decreased adherence over time with multiple follow-up visits making no significant difference. Many adjustments are required of the MI patient, especially in the first months postinfarction. Based on the literature, to be effective a nursing intervention needs to include physical, psychological, social, situational, and medical regimen variables to facilitate MRA and SA postinfarction.

This study was designed to examine the effects of a nursing intervention, which included data on medical regimen prescriptions, and the variables of attitudes, intentions, perceived beliefs of others, and coping methods on medical regimen adherence and societal adjustments postinfarction. The nursing intervention combined efforts of patient, spouse, and nurse to develop specific health goals that matched regimen restrictions and life situations encountered by patients attempting to return to their desired life-style within restrictions imposed on them by their infarction.

Hypotheses

- I. Medical regimen adherence and societal adjustment will increase more in patients in the nursing intervention group than in patients in the control group.
- II. Attitudes, intentions, perceived beliefs of others, and coping methods are predictors of societal adjustment and regimen adherence.

Method

Subjects: The subjects selected for this study were patients in one of three hospitals with comparable cardiac rehabilitation programs. Each subject had been diagnosed as having had a first myocardial infarction, and none had any major complications postinfarction. Their ages ranged from 30 to 65 years. While in the hospital, each patient completed a 10-15 day cardiac rehabilitation program that included instructions on cardiac condition and specific prescriptions related to diet, smoking, activity, medications, and stress management.

The initial sample included 115 subjects who were alternately assigned to the experimental condition ($n = 58$) or the control condition ($n = 57$). At the 30-day visit, there were 109 subjects ($E = 56$, $C = 53$), and at the 60-day visit, there were 103 subjects ($E = 56$, $C = 47$). Two subjects died (1E and 1C), six could not be located during the follow-up, and four did not wish to continue participation. Data analyses were completed for the 103 subjects participating at all three data collection periods.

Table 1 presents demographic characteristics of the sample at the time they were entered into the study. The

Table 1. Selected Demographic Characteristics of 103 Myocardial Infarction Patients

DEMOGRAPHIC VARIABLES	EXPERIMENTAL ($n = 56$)		CONTROL ($n = 47$)	
	N	(%)	N	(%)
Sex				
Male	41	(73)	42	(89)
Female	15	(27)	5	(11)
Race				
White	55	(98)	41	(87)
Black	1	(2)	6	(13)
Marital status				
Single			2	(4)
Married	42	(75)	37	(79)
Separated	2	(4)	2	(4)
Divorced	7	(12)	4	(9)
Widowed	5	(9)	2	(4)
Smoker				
Yes	35	(63)	24	(51)
No	21	(37)	23	(49)
Occupation				
Unknown	1	(2)	2	(4)
Retired	7	(12)	5	(11)
Laborer and service	14	(25)	11	(24)
Proprietor and skilled craftsman	22	(39)	15	(32)
Professional	10	(18)	11	(23)
Top management	2	(4)	3	(6)

experimental group had more female subjects than the control group. There were no significant differences between experimental and control groups for any demographic or medical variable. Subjects had normal blood pressure (systolic, $E, M = 123.4$ mmHg, $SD = 14.9$; $C, M = 123$ mmHg, $SD = 14.1$; diastolic, $E, M = 75$ mmHg, $SD = 8.9$, $C, M = 75$ mmHg, $SD = 10.7$). Eighty-two subjects ($E = 45$, $C = 37$) reported they had smoked at some time in the past with a mean of two packs per day. Sixteen ($E = 8$, $C = 8$) were smoking at the 30-day visit and 22 ($E = 12$, $C = 10$) at the 60-day visit. Subjects were overweight ($E, M = 185$ lbs; $C, M = 181$ lbs). This was further verified by calculating a Quetelet Index which accounts for height and weight. The index was 4.1 for the experimental group and 4.0 for the control group; normal is 2.7 to 3.3 (Kleinbaum & Kupper, 1978).

Spouses were included in the study for validation purposes and to participate in the intervention with the experimental group. Spouses included husband, wife, or significant other named by the patient as sharing responsibility for his or her well-being.

Instruments: MILLER ATTITUDE SCALE (MAS). The MAS (Miller, Wikoff, McMahon, Garrett, & Johnson, 1982b) is a 9-item, 7-point semantic differential scale to measure favorable or unfavorable attitudes toward performing regimen prescriptions. The same nine sets of bipolar adjectives are used for each of five prescriptions in the medical regimen (medication, stress, activity, smoking, and diet). The subscale mean score (possible 63 points) was used to measure attitude toward each regimen prescription. Reliability, content, and beginning construct validity of the scale has been established (Miller et al.). Alpha reliabilities of the five subscales for this sample were medication, .84; activity, .84; stress, .83; smoking, .86; and diet, .87.

HEALTH INTENTION SCALE (HIS). The HIS is a 5-point Likert scale designed to assess subjects' intentions to perform the medical regimen, such as, "If I am at home, I will follow my diet." The HIS has five subscales, one for each regimen prescription (diet, smoking, activities, med-

ications, and modification of stress response). There are items for each of four situations: home, work, sports, or social. Subscale mean scores (possible 20 points) were used to measure health intentions for each prescription. Scale development is described in detail by Miller et al. (1982a). Alpha reliabilities of the five HIS subscales administered during hospitalization for this study sample range from .73 to .97.

HEALTH BEHAVIOR SCALE (HBS). The HBS, a modification of the HIS, is a 5-point Likert scale (Miller et al., 1982a) which measures patient adherence to medical regimen (MRA). Intention statements were changed to behavior (adherence) statements. For example, "If I am at home, I will follow my diet" was changed to read "If I am at home, I follow my diet." Subscale mean scores (possible 20 points) were used to measure MRA for each prescription. Because validity of self-reporting is often questioned, the scale was completed independently by a family member for validation of patient MRA at the 30- and 60-day visits. Using *t* tests, no significant differences were found between patient and spouse reports of patient MRA, indicating there was agreement about subject adherence. Alpha reliabilities in this study for the five HBS subscales ranged from .82 to .92.

PERCEIVED BELIEFS OF OTHERS SCALE (PBOS). This is a 5-point Likert scale designed to assess subjects' beliefs about which prescriptions of the medical regimen persons most important to them think they should perform, such as, "Most people who are important to me think I should follow the prescribed diet at home." The PBOS has five subscales (diet, smoking, activities, medications, and modification of stress response) with items for each of the four situations of home, work, sports, and social. Total subscale mean scores (possible 20 points) were used as the measure of PBO. Scale development is described in detail by Miller et al. (1982a). Alpha reliabilities for the five PBO subscales for this study sample during hospitalization and at the 30- and 60-day visit range from .80 to .95.

GARRITY SOCIAL AND PSYCHOLOGICAL STATUS SCALE (GSPSS). This scale, designed to measure the subject's societal adjustment (SA), has six subscales: perception of health, activity level, sexual feelings, social life, worry, and morale. For each subscale, a 7-point Cantril ladder is used to measure the degree of adjustment for each variable. The total scale mean score (possible 42 points) is used as the measure of SA. Scale development and factor analysis have been reported, although no further testing for psychometric properties has been done (Garrity, 1979). Alpha reliability for this study sample was .74.

CARDIAC COPING SCALE (CCS). The CCS, a 5-point Likert scale with choices from *never* (1) to *always* (5) contains 12 specific coping methods. The CCS measures (a) specific coping methods and (b) helpfulness of the specific methods used. The frequency of each coping method used is determined and a total mean score is calculated to specify the overall helpfulness of coping methods. Scale development has been reported by Miller et al. (1985a). Alpha reliabilities for the CCS range from .77 to .83. Alpha reliability for this study sample was .78.

DEMOGRAPHIC AND MEDICAL DATA identified during hospitalization included age, height, sex, race, marital status, occupation, current employment, and education. Medical data obtained included weight, blood pressure, specific

medical regimen prescriptions, and history of heart disease and smoking. At 30 and 60 days posthospitalization, medical data obtained included medical regimen actions practiced and cardiac problems experienced. Subjects also rated helpfulness of the nursing visit on a 5-point Likert scale (*not helpful*, 1, to *very helpful*, 5).

Procedure: Consent was obtained from the university institutional review board, institutions, patients, spouses, and physicians. Baseline data on demographic and medical variables were collected and the MAS, HIS, PBOS, and CCS were administered to all subjects just prior to dismissal from the hospital. At 30 and 60 days posthospitalization, subjects were visited in their homes and reassessed on all the above instruments with the addition of the HBS and GSPSS. The HIS was not administered at the 30 and 60 day visits. The 30-day time period was chosen because it allowed the individual to resume life activities and to have experienced problems of adjustment at home, at work, and socially. The 60-day visit provided an opportunity to measure the effect of the nursing intervention on MRA and SA.

Two cardiovascular nurses were trained to visit both experimental and control patients. Preparation of the research nurses included acquiring knowledge of cardiac rehabilitation programs, training in interviewing skills, and familiarization with the study protocol. Practice sessions with nurses and volunteer patients were videotaped and evaluated for consistency of content and protocol.

EXPERIMENTAL GROUP (NURSING INTERVENTION). The nursing intervention involved three steps. First, data were obtained on the variables discussed above and compared with baseline data obtained during hospitalization. Second, baseline 30-day data were used for problem identification by discussing the cognitive and attitudinal information with the patient and spouse. The problems identified related to following each medical regimen prescription and the effect of the patient's perception of beliefs of others on their MRA and SA. In addition, specific societal adjustments and coping methods were examined for their effectiveness. Information presented during the hospital rehabilitation program that remained unclear was reviewed or provided if missing.

Based on problems identified in Step 2, alternative actions that met the criteria of a desired life-style within the limitation of the disease were explored with the patient and spouse. The resultant health plan included specific patient goals that addressed problems of attitudes, adherence, coping methods, and societal adjustment in different life situations. Sixty days after hospitalization, the subject was visited again and the same variables were reassessed.

All nurse-patient-spouse interactions were audiotaped and transcribed to a written format. Reliability checks yielded a 99% coverage of the three steps of the intervention and supported consistency among the nurse intervenors.

CONTROL GROUP. Control subjects completed the same scales as experimental subjects just prior to dismissal from the hospital and at a 30- and 60-day visit in the home. They received no nurse intervention and there was no discussion of medical regimen or problems experienced. No problems were observed that would require physician contact for either the control or experimental group.

Table 2. Means and Standard Deviations for Intentions and Medical Regimen Adherence of 103 Postmyocardial Infarction Patients

MEDICAL REGIMEN PRESCRIPTION	INTENTIONS				ADHERENCE BEHAVIORS							
	HOSPITAL				30 DAYS				60 DAYS			
	EXPERIMENTAL M	SD	CONTROL M	SD	EXPERIMENTAL M	SD	CONTROL M	SD	EXPERIMENTAL M	SD	CONTROL M	SD
Diet	16.55	2.89	16.10	3.48	14.20	3.65	14.70	2.39	14.82	3.22	14.57	3.40
Smoking	16.21	3.65	15.28	4.08	15.18	3.29	14.25	3.40	15.01	3.96	14.62	3.94
Activity	18.41	1.81	17.89	3.06	16.62	2.47	16.28	2.90	16.30	2.98	16.19	3.70
Stress	16.91	3.20	16.89	3.02	15.96	2.71	14.79	3.16	15.73	2.13	14.81	3.70
Medications	19.34	1.32	19.62	0.76	18.21	1.75	18.00	1.90	17.80	2.23	18.19	2.80

NOTE: Total possible score = 20. *N* = 57 for experimental group, 47 for control group.

Results

Hypothesis I was tested by analysis of variance using MRA and SA as dependent variables and group (nursing intervention or control) as the independent variable. An analysis was done for each of the prescriptions. Means and standard deviations for MRA are presented in Table 2 along with baseline data. The means and standard deviations for SA respectively were: 30 days (E, *M* = 25.7, *SD* = 5.8; C, *M* = 25.9, *SD* = 6.2) and 60 days (E, *M* = 27.1, *SD* = 6.7; C, *M* = 28.1, *SD* = 5.8). Total possible score was 42.

Hypothesis I was not supported. There were no significant differences between experimental and control groups for MRA for any of the prescriptions or for SA and 30- or 60-day visits and no significant interactions between groups over time. There was a significant decrease in mean scores from hospital to 30 days, *p* < .001, on all dependent and independent variables, but no differences from the 30- to 60-day visit.

Hypothesis II was tested using stepwise multiple regression analyses. Because there were no significant differences between groups, data were combined for these analyses. The predictor variables were attitude, intention, perceived beliefs of others, and helpfulness of coping methods. Analyses were computed using MRA for each of the prescriptions and SA as criteria. The analyses were done at 30 days and repeated at 60 days. The means and standard deviations for attitudes and PBO are shown for each of the prescriptions in Table 3 for baseline data during hospitalization and at 30 and 60 days. Table 4 presents the regression results for each of the prescriptions for the 30-day follow-up data, and Table 5 presents the same informa-

tion for the 60-day follow-up. The means and standard deviations for coping helpfulness were: 30 days, *M* = 3.49, *SD* = .10; 60 days, *M* = 3.50, *SD* = .12 (total possible score = 5).

Hypothesis II was partially supported. Perceived beliefs of others were found to be predictive of MRA for smoking, activity, stress, and medication prescriptions at the 30-day visit and for diet, smoking, activity, stress, and medication prescriptions at the 60-day visit. Attitudes were predictive of medical regimen adherence for diet, smoking, and medication prescriptions at the 30-day visit and diet, smoking, stress, and medication prescriptions at the 60-day visit.

Intentions during hospitalization were predictive of MRA to diet, smoking, and activity prescriptions at the 30-day visit and diet and stress prescriptions at the 60-day visit. Helpfulness of coping methods was predictive of MRA to the prescription of stress modification at the 60-day visit. No other variable assessed during hospitalization was predictive of MRA or SA at 30 or 60 days. Only attitude toward the activity prescription was predictive of SA at either the 30- or 60-day visit.

In addition to the hypotheses, adherence in different settings and the helpfulness of the nursing visit were explored. Home was the setting of greatest adherence and work the least adherence at the 60-day visit. Mean scores in descending order were: home, E = 20.9 and C = 20.3; sports, E = 20.7 and C = 19.9; social, E = 20.5 and C = 19.9; and work, E = 17.6 and C = 18.3. For helpfulness of the nursing visit, there was a significant difference between the experimental (*M* = 4.36) and control group (*M* = 3.46), *p* < .001.

Table 3. Means and Standard Deviations for Health Attitudes and Perceived Beliefs of Others Toward Medical Regimen Adherence for 103 Myocardial Infarction Patients

MEDICAL REGIMEN PRESCRIPTION	HOSPITAL				30 DAY				60 DAY			
	EXPERIMENTAL M	SD	CONTROL M	SD	EXPERIMENTAL M	SD	CONTROL M	SD	EXPERIMENTAL M	SD	CONTROL M	SD
Diet												
Attitude	49.07	9.03	50.64	9.05	47.39	10.15	49.45	10.97	48.00	8.98	49.55	9.97
PBO	18.00	3.10	17.53	3.29	16.55	3.04	16.15	2.90	16.70	3.08	16.19	3.31
Smoking												
Attitude	44.54	9.32	42.53	9.86	44.86	10.00	42.72	9.22	44.32	9.60	43.83	11.07
PBO	16.75	3.77	15.55	3.75	15.96	3.31	15.06	3.51	16.30	3.49	14.91	4.10
Activity												
Attitude	55.96	7.25	55.96	6.98	55.09	6.52	54.25	7.31	53.87	8.01	52.81	8.24
PBO	19.25	1.25	18.85	2.23	17.70	2.03	17.90	1.98	18.05	2.10	17.74	3.09
Stress												
Attitude	52.52	8.14	53.21	8.37	49.23	8.88	50.77	9.49	50.79	8.17	52.32	8.15
PBO	15.59	2.85	18.34	2.59	17.62	2.07	17.08	2.06	18.11	1.94	17.17	2.92
Medication												
Attitude	59.86	5.42	59.21	4.68	56.21	8.06	57.68	6.23	57.05	7.26	57.49	6.98
PBO	19.36	1.33	19.43	1.12	17.96	2.51	18.49	1.46	18.00	2.57	18.34	2.65

NOTE: Total possible scores: Attitude = 63 and Perceived Belief of Others (PBO) = 20.
N = 56 for the experimental group, 47 for the control group.

Table 4. Regression Equations for Adherence with Perceived Beliefs of Others and Attitudes (30 Days Posthospitalization) and Intentions (in Hospital)

PRESCRIPTION	<i>a</i>	BETA			<i>R</i> ²	<i>F</i>
		PERCEIVED BELIEFS OF OTHERS	ATTITUDES	INTENTIONS		
Diet	4.31		.36	.32	.39	22.12*
Smoking	0.68	.46	.37	.18	.80	89.84*
Activity	4.33	.44		.26	.32	16.04*
Stress	9.47	.36			.22	9.63*
Medication	5.80	.57	.30		.54	40.36*

**p* < .000

Discussion

In this study of the influence of a posthospitalization nursing intervention on medical regimen adherence and societal adjustment of MI patients the nursing intervention was not found to facilitate MRA or SA postinfarction as had been hypothesized. In addition, neither favorableness of attitude nor perceived helpfulness of coping methods were affected, as had been expected by the nursing intervention.

These findings were in direct contrast to the predictions of the Fishbein model. The lack of sensitivity of instruments to measure MRA and SA may have accounted for no difference between groups. Because means for attitudes and adherence were very high, there may not have been enough variance to demonstrate differences. Also, scores for the control group may have been increased by the visit of the nurses for data collection, or by self-evaluation after completing the scales at 30 and 60 days even though no direct intervention procedure was followed.

MI patients participating in the nursing intervention indicated they believed the nurse's visit had been very helpful. They expressed positive values of the intervention in areas of problem adjustment, clarification of prescriptions, providing missing regimen information, problem solving, and goal setting. These values were not measured in this study, but suggest the need for further study. Considering that adherence and attitudes were strong for both groups, it may be that the worth of the nursing intervention lies in values as described by the patients.

Because mean scores decreased significantly for all variables for both the experimental and the control groups from the hospital to the 30-day visit with no further decline at the 60-day period, it appears that major societal adjustments are dealt with by the patient during the first month postinfarction. Therefore, this time period seems optimal for the health team to assist the patient in necessary life adjustments. Assessment of continued medical

regimen adherence and societal adjustment at periods beyond 60 days warrants further investigation.

Several relationships were found which are important for rehabilitation planning. Perceived beliefs of others was the strongest predictor of MRA for all regimen prescriptions. These results parallel those of Dhooper (1984), Holm et al. (1985), and Miller et al. (1985b) and are in the direction predicted by Ajzen and Fishbein (1980). These findings focus on the need to include significant others in the entire cardiac rehabilitation process.

In addition to PBO and intentions during hospitalization, attitudes and helpfulness of coping methods were predictive of adherence to the prescription for modification of the stress response at 60 days. The health team could assist rehabilitating MI patients by examining type and effectiveness of their coping methods and provide information about specific coping methods other MI patients have found helpful in resolution of similar problems. This would allow individualized modification of the stress response and may increase adherence to this prescription of the medical regimen.

For societal adjustment, only attitude toward the activity regimen prescription was predictive. Patients regarded their adjustment to activities as good. These findings, coupled with modern culture's emphasis on benefits of activity for cardiovascular health, should promote patient motivation to comply with the activity prescription.

Attitudes were most favorable and MRA strongest toward taking medications and weakest toward stopping smoking and following the diet. In addition, attitudes were found predictive of MRA for these same prescriptions. Smoking and diet prescriptions apparently need special consideration in rehabilitation programs. Only 16 subjects were smoking 30 days posthospitalization. However, this number increased to 22 by 60 days. Analysis of responses from the MAS indicated subjects believed stopping smoking was worthless, bad, and difficult. These findings are similar to Miller et al. (1982b) and are not hopeful for long-term adherence. Because attitudes according to the Fishbein model (Ajzen & Fishbein, 1980) are predictors of behavior, this may explain difficulties in long-term adherence to stopping smoking.

From qualitative data on diet prescriptions reported by subjects, new foods, lack of understanding, and absence of desire to adhere were major problems. For both smoking

Accepted for publication February 25, 1988.

This research project was partially supported by the National Institutes of Health under Grant Number 5 RO1 NR1057-03.

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Table 5. Regression Equations for Adherence with Perceived Beliefs of Others, Attitudes and Coping (60 Days Posthospitalization) and Intentions (in Hospital)

PRESCRIPTION	<i>a</i>	BETA			<i>R</i> ²	<i>F</i>
		PERCEIVED BELIEFS OF OTHERS	ATTITUDES	COPING INTENTIONS		
Diet	0.87	.32	.38		.71	33.09*
Smoking	3.16	.49	.45	-.13	.80	73.72*
Activity	6.40	.63			.50	27.69*
Stress	-5.93	.50	.25	.29	.40	18.82*
Medication	5.78	.45	.41		.48	25.40*

**p* < .000

and diet prescriptions, rehabilitation plans need to include strategies that combine informational and attitudinal components if long-term adherence is to be achieved. In addition, situational differences must be addressed. Work was the setting of lowest adherence in this study, findings similar to McMahon et al. (1986) and Taylor et al. (1986). The fact that 66 of the original 91 employed subjects in this study had returned to work at the 60-day visit is evidence that necessary regimen adjustments specific to the work setting need to be explored with the patient posthospitalization.

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